

1.1 Mental Health and Mental Illness

LEARNING OBJECTIVES

By the end of this section, you will be able to:

- Compare and contrast psychiatric-mental health to psychiatric-mental illness
- List standard nursing practices in psychiatric mental health

It is important for nurses to understand the line between mental health and mental illness. The human body has physical and psychological responses to stress that psychiatric-mental health nurses are trained to observe in their clients. The nurse's role is to assess, plan, implement, and evaluate, all while collaborating with the client. Collaboration between the nurse and the client is an essential part of the mental health-care nursing process.

Psychiatric-Mental Health versus Psychiatric-Mental Illness

The term **mental health** refers to a state of well-being in which individuals realize their own abilities, cope with the normal stresses of life, work productively, and contribute to their community. Mental health is an essential component of overall health. The World Health Organization (WHO) defines overall health as a state of complete physical, mental, and social well-being and not merely the absence of disease or infirmity. Conversely, according to the American Psychiatric Association, **mental illness** is a health condition involving changes in emotion, thinking, or behavior (or a combination of these) associated with emotional distress and problems functioning in social, work, or family activities (American Psychiatric Association, n.d.).

It is important that “health services . . . devote as much attention to mental health as they do to physical health care” (Happell et al., 2021, p. 560). The World Health Organization’s slogan agrees: “There is no health without mental health.” Yet attitudes toward choosing mental health nursing as a specialty are ambivalent. Nurses should receive education that reduces stigma surrounding mental health and its treatment, increases their knowledge about mental health/illness, and exposes them to caring for clients with mental health problems in all nursing settings.

As a note, medical orientation to mental health treatment may refer to recipients of care as *patients* where the person is receiving acute treatment or assistance from a medical provider. *Client* is more often used in the community setting in which a person is in a collaborative solution-based relationship with a therapist (Spector, 2016). In the 1990s, nurses began to use the term *client* more regularly in an effort to show that the person was working in collaboration with their health-care team (American Psychiatric Nurses Association [APNA], 2022). This text will, for the most part, use the word *client*.

Psychiatric-Mental Health

The promotion, protection, and restoration of mental health is a vital concern of individuals, nurses, communities, and societies throughout the world (Figure 1.2).

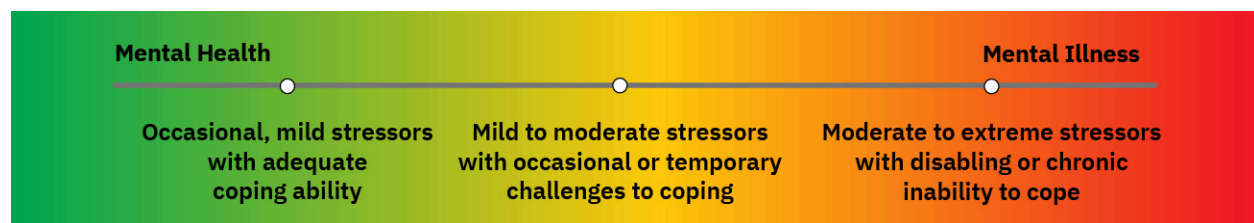


FIGURE 1.2 Mental health and mental illness exist on a continuum. (attribution: Copyright Rice University, OpenStax, under CC BY NC-SA 4.0 license)

Mental health fluctuates over the course of an individual’s life span and can range from well-being to emotional problems and/or mental illness, as indicated on the mental health continuum. Well-being is on the “healthy” range of the mental health continuum, where individuals are experiencing a state of good mental and emotional health. They may experience stress and discomfort resulting from occasional problems of everyday life, but they are able to cope effectively with the stressors and experience no impairments to daily functioning.



PSYCHOSOCIAL CONSIDERATIONS

Stress and Physical Response

The body's response to stress, often called the fight-or-flight response, can initiate both physical and psychological symptoms. The fight-or-flight response is triggered when a stressful situation puts hormones into overdrive. Even if the stress is not life-threatening, this response has evolved over the years as a survival mechanism against situations deemed dangerous (Harvard Health Publishing, 2020). This response can entail a person sweating, enduring chest pain, panic, stomach upset, headache, and an overall sense of doom. Chronic stress can lead to high blood pressure, insomnia, anxiety, depression, poor appetite or overeating, and substance use. Stress, of course, does not equal mental illness.

Psychiatric-Mental Illness

Mental illness is common in the United States. Nearly one in five (19 percent) of adults experience some form of mental illness, one in twelve (8.5 percent) have a substance use disorder, and one in twenty-four (4 percent) have a serious mental illness (American Psychiatric Association, 2022). Poor mental health increases the risk of chronic physical illnesses, such as heart disease, cancer, and strokes, and can lead to thoughts and intentions of suicide. Suicide is a common symptom associated with mental illness and is the second leading cause of death in Americans aged fifteen to thirty-four (Centers for Disease Control and Prevention, 2021a).

Emotional problems become classified as *mental illness* when an individual's level of distress becomes significant, and they have moderate to severe impairment in daily functioning at work, school, or home. Mental illness includes relatively common disorders, such as depression and anxiety, as well as less common disorders, such as schizophrenia. Mental illness is characterized by alterations in thinking, mood, or behavior. The term serious mental illness refers to that which causes disabling functional impairment that substantially interferes with one or more major life activities.

Standards of Psychiatric-Mental Health Nursing Practice

The American Psychiatric Nurses Association (APNA), the International Society of Psychiatric-Mental Health Nurses (ISPN), and the American Nurses Association (ANA) have established standards of care for psychiatric-mental health nursing practice. These standards include that individuals with mental health and substance use conditions should be treated with respect and dignity in a culturally appropriate manner. Health-care professionals should consider the preferences of people with mental health and substance use disorders and support them, their family members, and their loved ones in an inclusive manner.

History of Psychiatric Nursing

In the late 1800s, Edward Cowles, a physician at McLean Asylum in Massachusetts, began the first program to train nurses to care for psychiatric clients (APNA, 2022). Prior to that, the caregivers were called “keepers” (p. 8). In the early 1900s, Effie Jane Taylor at Johns Hopkins Hospital organized the first nurse-taught course to train psychiatric nurses. Before that time, physicians trained nurses. The first psychiatric nursing textbook was published in 1920 (p. 8). World War II expanded the need for psychiatric nurses as veterans returned from war with combat-related mental health problems.

It was in the 1950s that Hildegard Peplau rose to prominence as the “mother of psychiatric nursing” with her development of the theory of interpersonal relations, a theory and model of the therapeutic nurse-client relationship that focused on the “therapeutic use of self in promoting the well-being of individuals, families, groups, and communities” (Haber, 2000, p. 56) ([Figure 1.3](#)). Her theory not only helped nurses to become self-aware in their therapeutic relationships, but also assisted clients to build autonomy in problem-solving due to the support they received from the nurse while moving forward toward better mental health. Peplau's mission was to redefine the scope of work of the psychiatric nurse as a collaborative part of the health-care team and not just as the physician's “handmaiden” (p. 57).



FIGURE 1.3 Hildegard Peplau's model for the therapeutic nurse-client relationship shows what the nurse and the client bring to the therapeutic relationship. (attribution: Copyright Rice University, OpenStax, under CC BY NC-SA 4.0 license)



LINK TO LEARNING

The National Council of State Boards of Nursing created the [Clinical Judgment Measurement Model \(CJMM\)](https://openstax.org/r/77CJMM) (<https://openstax.org/r/77CJMM>) to explore new ways of testing clinical judgment in nursing as part of the National Council Licensure Examination (NCLEX). The diagram at this site shows the layers of the model.

CLINICAL JUDGMENT MEASUREMENT MODEL

What Is the Clinical Judgment Measurement Model (CJMM)?

Designed by the National Council of State Boards of Nursing (NCSBN), the CJMM measures a nursing licensure candidate's ability to exercise sound clinical judgment and decision-making. Grounded in the nursing process, the CJMM includes six components: (1) recognize cues, (2) analyze cues, (3) prioritize hypotheses, (4) generate solutions, (5) take actions, and (6) evaluate outcomes. Built in layers, the CJMM provides a framework for thought processes and actions to take in exercising clinical judgment in a nursing scenario.

Current Scope of Psychiatric Nursing Practice

All nursing practice is based on theory and a caring philosophy toward the provision of holistic care. Psychiatric-mental health nursing is a specialty practice focused on the client's psychological and emotional responses, level of risk, and coping abilities. The client's recovery is the goal, with individualized care as the process. The therapeutic nurse-client relationship is key to this process.

Mental health nursing takes place across settings, in the emergency room, in the community, in schools, in jails, in medical offices, in person, and through telehealth. Psychiatric nurses not only assess and provide care for the client's health-care needs, but are also involved in education, administration, and research (APNA, 2023, p. 40). When evaluating a client's mental health, the nurse uses a variety of assessments in addition to the traditional physical examination:

- performing a mental status examination
- completing a psychosocial assessment
- reviewing the client's use of psychotropic medications (drugs that treat psychiatric symptoms) and/or other medications that can cause psychiatric symptoms as side effects
- screening for suicidal ideation, exposure to trauma or violence, and substance misuse
- incorporating a spiritual assessment while assessing the client's coping status
- incorporating life span, developmental, and cultural considerations
- reviewing specific laboratory results related to the client's use of psychotropic and other medications



LINK TO LEARNING

The [full scope of psychiatric nursing](https://openstax.org/r/77fullscope) (<https://openstax.org/r/77fullscope>) can be accessed on the American

Psychiatric Nurses Association website.

Nursing assessments related to mental health disorders differ from physiological assessments because they have a greater focus on collecting subjective data: information provided by the client from the client's point of view or a description of their experience. For example, prior to administering a cardiac medication to a client with a heart condition, a nurse will assess objective data, such as blood pressure and an apical heart rate, to determine the effectiveness of the medication treatment.

Prior to administering an antidepressant, however, a nurse uses therapeutic communication to ask questions and gather subjective data about how the client is feeling in order to determine the effectiveness of the medication. The nurse will also observe client behaviors, speech, mood, and thought processes as part of the assessment. Nurses cannot directly measure a neurotransmitter to determine the effects of an antidepressant, for instance, but they can ask questions to determine how the client is feeling emotionally and perceiving the world, two factors influenced by neurotransmitter levels. An example of a nurse using therapeutic communication to perform subjective assessment is, "Tell me more about how you are feeling today." The nurse may also use general survey techniques, such as simply observing the client, to assess for cues of behavior. Examples of data collected by a general survey could be evaluating the client's mood, hygiene, appearance, or movement.

Nurses in any setting holistically observe and process their clients' physical, emotional, and mental health, as well as any impairments affecting their functioning. They must recognize subtle cues of undiagnosed or poorly managed physical and mental disorders and follow up appropriately with other members of the interprofessional health-care team (Figure 1.4).



FIGURE 1.4 The QSEN model includes six competencies, along with knowledge, skills, and attitudes (KSAs) for each competency. (credit: "Core Competency Framework for Undergraduate Nursing Student" by Dena Attallah and Abd Alhadi Hasan/Nursing Reports, CC BY)



CLINICAL SAFETY AND PROCEDURES (QSEN)

Competency: Description of QSEN Competencies

The references to Quality and Safety Education for Nurses (QSEN) competencies throughout this text refer to the educational repository created through funding by the Robert Wood Johnson Foundation to provide evidence-based teaching resources for nursing curricula. This was in response to the Institute of Medicine's call to improve the quality of healthcare in its report on healthcare safety in 1999. QSEN competencies are aligned with concepts that

can be expected to be tested on the NCLEX Next Gen, and with concepts presented in National Patient Safety Goals published annually by the Joint Commission.

The overall goal of QSEN is to prepare future nurses with the knowledge, skills, and attitudes (KSAs) necessary to continuously improve the quality and safety of the health-care systems within which they work. QSEN helps nurses to identify and bridge the gaps between nursing school and nursing practice. QSEN includes six core QSEN competencies, KSAs, teaching strategies, and faculty development resources. The competencies arose after the Institute of Medicine (IOM) released a report in 2000 that highlighted the need for health-care system redesign. The six QSEN competencies are:

- patient-centered care
- teamwork and collaboration
- evidence-based practice (EBP)
- quality improvement (QI)
- safety
- informatics

These competencies align with both the components of the NCLEX and the Joint Commission 2024 Behavioral Health Care National Patient Safety Goals (Table 1.1). The QSEN Competency of EBP aligns with the NCLEX Integrated Process of clinical judgment.

QSEN Competencies	Components of NCLEX Test Plan 2023, Client Needs	The Joint Commission 2024 Behavioral Health Care National Patient Safety Goals
Quality improvement (QI) Safety Informatics	Safe and Effective Care Environment Management of Care Safety and Infection Control Reduction of Risk Potential	Identify individuals served correctly Use medicines safely Prevent infection
Patient-centered care Teamwork and collaboration	Health Promotion and Maintenance Psychosocial Integrity Physiological Adaptation	Identify individuals served safety risks Improve health care equity

TABLE 1.1 QSEN Competencies (Sources: Ferro & Yoder, 2023; Joint Commission, 2024; QSEN Institute, 2022; Stanley et al., 2023)

1.2 Risk and Protective Factors of Mental Health

LEARNING OBJECTIVES

By the end of this section, you will be able to:

- Discuss various risk factors of psychiatric-mental illness
- Describe various protective factors for psychiatric-mental illness
- Identify nursing implications associated with risk and protection factors

A **risk factor** increases the chances of developing a mental illness, may determine the probability of having a psychiatric-mental illness, and can change through the life span. Some risk factors may be modifiable, while others may be genetic or biological. Both internal and external environments of the person may pose risk. All of these factors can be addressed in nursing assessment. Having risk factors does not necessarily mean that a client will develop a disorder, and nurses shouldn't assume that a client is experiencing or will develop a disorder or illness. In fact, although risk factors can increase the chances of developing a mental illness, having an array of protective factors can decrease that risk. Nurses should help clients understand what these risk and protective factors are and potential ways to minimize risk factors and maximize protective factors.

Risk Factors

Mental health researchers have developed several theories to explain the causes of mental health disorders, but

they have not reached consensus [Figure 1.5](#). One point on which they agree is that an individual is not at fault for the condition; they cannot simply turn symptoms on or off at will. There are likely several environmental, biological, and genetic risk factors that, when combined, trigger a mental health disorder (University of Michigan Human Resources, n.d.).

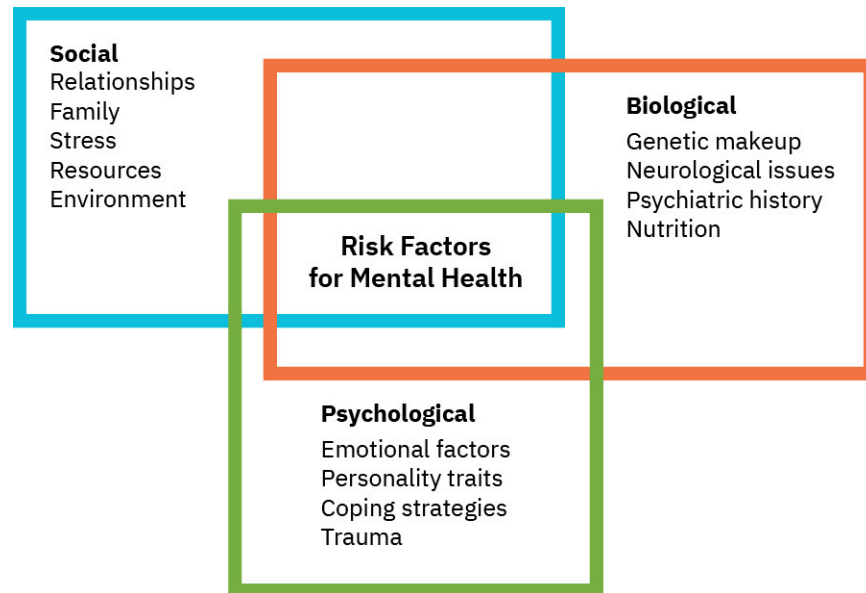


FIGURE 1.5 Risk factors for psychiatric-mental health conditions include social, biological, and psychological factors. (attribution: Copyright Rice University, OpenStax, under CC BY NC-SA 4.0 license)

Risk factors from biological, social, and psychological factors can overlap. Some risk factors change over time, such as age, health history, availability of resources, and psychiatric history. Others are considered variable risk factors and include “income level, peer group, adverse childhood experiences (ACEs), and employment status” (SAMHSA, n.d.-a). Current stressors, such as relationship difficulties, the loss of a job, the birth of a child, a move, or prolonged problems at work can also be important contributory environmental factors (University of Michigan Human Resources, 2023).

Internal Environment: Physiological

Current research shows that the brain can have an imbalance of neurotransmitters, such as dopamine, acetylcholine, gamma-aminobutyric acid (GABA), norepinephrine, glutamate, and serotonin, resulting in changes in behavior, mood, and thought. While causes of fluctuations in brain chemicals aren’t fully understood, contributing factors can include physical illness, hormonal changes, reactions to medication, substance misuse, diet, and stress (University of Michigan Human Resources, 2024). Organic delirium and infections can also cause changes to mood and behavior, until these conditions are successfully treated.

Kraybill (2019) found research suggesting that some mental health disorders are accompanied by immune system dysregulation and inflammation. The immune system can be disturbed by stress and environmental toxins, affecting human responses, nutritional intake, and immunity. Inflammation develops in the body as a response. Inflammation is also a trigger for depression (Kraybill, 2019).

Genetics and Medical Comorbidities

There appears to be a hereditary pattern to some mental illnesses. For example, individuals with major depressive disorder often have parents or other close relatives with the same illness. Huntington disease is another example, wherein the cognitive and motor impairment can be misdiagnosed as a psychiatric disorder. Researchers continue to investigate genes involved in specific disorders in order to target treatment to individuals.

People with mental illness may also have medical comorbidities, such as diabetes, endocrine dysfunction, Alzheimer’s disease, cancer, and traumatic brain injuries (TBI) (American Mental Wellness Association, 2023). Psychiatric symptoms, medication side effects, ability for self-care, and provider focus may influence overall health.

Intrapartum Complications

People with a history of mental health issues prior to becoming pregnant are at a 40 percent higher risk of having a complication during pregnancy, a 50 percent higher risk for a non-live birth, and double the risk for having a baby born with a low birth weight (Witt et al., 2012). Some of these problems are due in part to lower socioeconomic status, lower education level, lack of access to medical care, and not being adequately followed to check their mental health status during pregnancy.

Moreover, Ross et al. (2015) reports that drug use among pregnant females aged fifteen to forty-four is at nearly 6 percent. Though the effects of maternal drug use on the babies differ depending on the drug used, some of the more common mental health effects on babies include cognitive delays, behavioral problems, attention-deficit hyperactivity disorder (ADHD), increased impulsivity, anxiety, depression, future substance use, and adolescent aggression (Table 1.2). Legal substances also are an intrapartum risk factor for mental illness. Tobacco exposure while in utero can cause children to develop ADHD, conduct disorder, behavioral disorders like aggression, and increased risk of drug abuse problems later on (Ross et al., 2015). Alcohol use in pregnancy can lead to fetal alcohol syndrome (FAS), which causes “morphogenic effects on limb and facial development, reduced brain and birth weight, and cognitive delays and impairments” (p. 74).

Drug	Effects
Cannabinoids	Attention deficits Future substance abuse Future depression Poor growth Long-term problems with executive function
Caffeine	Increased chance of prematurity Problems in executive function once in school
Psychostimulants	Preterm labor Behavioral problems Attention deficits Brain abnormalities Cardiac anomalies Aggression Anxiety/depression
Alcohol	Decreased growth Attention deficits Prematurity or spontaneous abortion Cognitive delays Brain abnormalities Problems with limb and facial development

TABLE 1.2 Effects of Maternal Drug Use

Drug	Effects
Tobacco	Lower birthweight ADHD Poor academic performance Aggression in adolescence Oppositional defiance
Opiates	Neonatal abstinence syndrome Preterm birth Lower weight Heart defects Low IQ Behavioral problems Respiratory problems

TABLE 1.2 Effects of Maternal Drug Use

Substance Use Disorders and Addictions

In addition to the negative effects that substance use and addiction have on the developing fetus, about half of the people living with a mental health disorder are also living with a substance use disorder (Medline Plus, 2019). Each of these things, mental health disorders and substance use disorder, can contribute to the development of the other. Many people who have mental illness use drugs or alcohol to cope with their symptoms.

Brain Injury

Another internal physiological risk factor for mental illness that can affect children and adults is experiencing a **traumatic brain injury (TBI)**, a brain injury that can cause short- or long-term problems with thinking, physical movement, communication, and functional abilities. In some cases, it may result in permanent disability or death. A TBI “can be caused by a forceful bump, blow, or jolt to the head or body, or from an object that pierces the skull and enters the brain” (National Institute on Neurological Disorders and Stroke, n.d., para 1). According to the Centers for Disease Control and Prevention (2022), a TBI in children can affect their development and lead to problems with learning, thinking, and behavior. These effects can last a lifetime. The most common causes of TBIs are falls, firearm injury, car accidents, and assaults. Experiencing a TBI can lead to years of medical care, rehabilitation, PTSD from memories of the event, a decrease in physical and mental functioning, financial concerns, and mental health issues for partners or family members (Tsur & Haller, 2020).

Internal Environment: Psychological

Factors that affect a person’s psychological self include their emotions, coping ability, spirituality, self-concept and self-esteem, learned and conditioned behaviors, and personality traits. Plus, individual trauma resulting from an event, series of events, or set of circumstances that is experienced as physically or emotionally harmful can have lasting adverse effects on the individual’s functioning and mental, physical, social, emotional, or spiritual well-being (U.S. Department of Health and Human Services, 2020).

Low Self-Esteem, Poor Self-Concept

Having a poor self-concept or low self-esteem can lead to a person having negative thoughts about themselves. These feelings can lead to a sense of isolation and can cause poor relationships with others. Continued negative thoughts can lead a person to use substances as they try to make themselves feel better. Low self-esteem can also be associated with depression and anxiety (Gold, 2016).

Dysfunctional Relationships and Trust Violations

A relationship that is characterized as a **dysfunctional relationship** is one in which unhealthy behavior patterns exist among members of a group. These patterns can include poor communication styles, conflict between people in the relationship, and even emotional and/or physical abuse (Harold, 2023). In families, while it is often the parent who engages in the dysfunctional behavior, it is the child who suffers the consequences with an increased risk of developing anxiety, low self-esteem, post-traumatic stress disorder (PTSD), substance misuse, depression, and

problems building healthy relationships. In dysfunctional families, children can lose their ability to trust others based on their experiences of not being able to depend on their parents (Martin, 2018).

One example of dysfunctional relationships is gang affiliation. Gang affiliation is an unsafe environment that worsens existing problems. Children brought up in a family setting that includes a parent who uses substances, is abusive, or is a gang member themselves are at greater risk of socializing with gang members they meet in school or on the streets (Macfarlane, 2018). As children push away from these families, their vulnerability draws them toward gang membership. Research has shown that high rates of “depression, psychosis, anxiety, alcohol and drug dependence, antisocial personality disorder and history of attempted suicide” (p. 414) occur in gang members.

External Environment: Social

Individuals are affected by broad social and cultural factors, as well as by unique factors in their personal environments. Social risk factors, such as racism, discrimination, poverty, and violence (often referred to as “social determinants of health”) can increase the chances of a person developing a mental illness.

While many social relationships act as sources of support, there are situations where relationships are toxic and can potentially increase the risk of mental health problems; fatigue; lack of motivation; difficulty concentrating; and cardiovascular, immune, and endocrine problems (André & Baumeister, 2023). Marital problems, demanding or intense relationships, and caregiving burnout are examples of relationships that could be toxic.



CULTURAL CONTEXT

Cultural Influences on Mental Health and Mental Illness

The culture of a client influences many aspects of mental health and mental illness. Be mindful, however, that general statements about cultural characteristics of a given group may also invite stereotyping. Cultural information should not be broadly applied to any individual member of a racial, ethnic, or cultural group.

In fact, only 66 percent of adults from underrepresented groups in the United States will get help for their mental health problems. There are four ways that culture affects mental health: stigma, understanding symptoms, lack of support from others in the same culture, and not having resources available that meet cultural needs (Mental Health First Aid, 2019).

Abuse/Neglect, Social Oppression/Victimization

In 2019, there were 656,000 victims of child abuse and neglect, with a victim rate calculated as nine victims per 1,000 children across the United States. More specifically, 74.9 percent of victims were neglected, 17.5 percent were physically abused, and 9.3 percent were sexually abused (Administration for Children & Families, 2021). Neglect is a situation in which a parent or caretaker fails, refuses, or is unable, for reasons other than poverty, to provide the necessary care, food, clothing, or medical or dental care, seriously endangering the physical, mental, or emotional health of the child. Physical abuse is defined as injury inflicted on a child by other than accidental means. Physical injury includes, but is not limited to, lacerations, fractured bones, burns, internal injuries, severe or frequent bruising, or great bodily harm. Sexual abuse is defined as sexual intercourse or sexual touching of a child; sexual exploitation; human trafficking of a child; forced viewing of sexual activity; or permitting, allowing, or encouraging a child to engage in prostitution. Emotional abuse is defined as harm to a child’s psychological or intellectual functioning, which often results in severe anxiety, depression, withdrawal, or aggression. Emotional damage may show up in substantial and observable changes in behavior, emotional response, or learning that are incompatible with the child’s age or stage of development. All of these types of abuse are considered adverse childhood experiences (ACEs).

It is estimated that 61 percent of adults have experienced early ACEs, such as abuse, neglect, or growing up in a household with violence, mental illness, substance misuse, incarceration, or divorce. Chronic stress from ACEs can change brain development and affect how the body responds to stress. ACEs are linked to chronic health problems, mental illness, and substance misuse in adulthood (National Center for Injury Prevention and Control, Division of Violence Protection, 2022).

Adults at risk for abuse are defined as adults who have a physical or mental condition that impairs their ability to

care for their own needs (Wisconsin Department of Health Services, 2018). Older adults at risk are potentially susceptible to abuse, neglect, or financial exploitation by caregivers or a person they trust (Centers for Disease Control and Prevention, 2021b).



LIFE-STAGE CONTEXT

Older Adult Risk Factors for Psychiatric-Mental Illness

Risk factors for mental disorders in older adults include being female, loneliness, alcohol abuse, lack of education, financial difficulties, family history of mental diseases, and severe physical disease. In addition, adults with serious mental illness over age fifty have a high rate of medical comorbid conditions (SAMHSA, 2021).

Social oppression and victimization—bullying, violence, and stigmatization of people from underrepresented groups, for instance—can occur across the life span and are also significant risk factors for mental illness. It is **stereotyping** when someone has a generalized and prejudiced opinion about members of a particular group of people.

Stereotyping can cause the marked population to feel stigmatized, disrespected, and become the target of bullying. Atunah-Jay et al. (2022) studied the concepts of school-based and electronic bullying in relation to their connection to an increase in suicidality in a group of North Dakota middle school students, specifically within the American Indian/Alaska native (AI/AN) population. Results revealed that “compared to other forms of bullying, electronic bullying is associated with unique and significant negative mental health concerns, including anxiety, depression, and suicidality” (p. 28).

Lack of a Support System, Loss, Economic Disadvantage

Risk factors for mental illness include the lack of a support system. This could be because the person is not physically or emotionally close to family members, has few friends, or lives in an area in which they do not feel connected to others. Loss includes anything that creates a negative effect for a person—loss of a spouse (divorce or death), loss of a job, and moving away from family are examples. Economic disadvantage can be related to the loss of a job or not having the skills to work beyond a minimum wage job. While lack of a support system, loss, and economic disadvantage have always been risk factors for developing mental health issues, the recent COVID-19 pandemic, and the measures taken to mitigate the spread, highlighted new examples of these risks. In particular was the practice of social distancing and social isolation. While these practices began to keep the public physically safer, there was a marked increase in anxiety, depression, and self-perceived stress related to loss of contact with others, loss of family and friends to the illness, sleep difficulties, financial insecurities among those with lower incomes, and misinterpretation (fake news) of the vast amount of information shared in mass media (Daclan, Ferreira, & Guzella, 2022).



LINK TO LEARNING

The Mayo Clinic provides [a list of mental illness risk factors \(https://openstax.org/r/77riskfactors\)](https://openstax.org/r/77riskfactors) with more information about mental illness symptoms, causes, treatment, and resources.

Protective Factors

There is a link between protective factors and resilience, or the ability to “bounce back,” and overcome stressful barriers to living a healthy life. Protective factors reduce the risk for mental illness, suicide, and other detrimental behaviors. Some of these biological, social, and psychological factors can be developed over time in the individual, family, and community (Connect Online, 2023). Examples of protective factors include support received from family and friends, religious practices, being part of a social group, getting physical activity, eating healthy foods, and using positive coping skills (Mental Health First Aid, 2022).

Internal Environment: Physiological

Living a healthy lifestyle that includes attention to diet, getting plenty of exercise and sleep, and having access to regular medical care provides a protective factor against the development of a mental illness. Other protective factors include avoiding high-risk substance use and receiving appropriate care for medical conditions (American

Mental Wellness Association, 2023).

Exercise is known to reduce all mortality risks, including poor mental health (Prakash, 2020). “Evidence suggests an exercise duration of 45 min 3–5 days a week to be optimal for good mental health” (p. 186). Physical activity is an appropriate intervention for the prevention of depression symptoms. Diet is also known to affect mental health, either positively or negatively. What a person eats can influence the development and progression of mental illness. Mental health professionals are encouraged to address nutrition in client care (Prakash, 2020).



LINK TO LEARNING

Dietary antioxidants may help to alleviate depression, anxiety, and stress. A [study of the Mediterranean and DASH diet \(https://openstax.org/r/77MedDASH\)](https://openstax.org/r/77MedDASH) investigates the relationship between diet and psychological disorders.

Internal Environment: Psychological

Other protective factors include having healthy development and good attachment to parents (American Mental Wellness Association, 2023). Fei, et al. (2021) found that secure attachment creates comfort with self and was associated with higher levels of self-esteem.

Moreover, having high self-esteem means that a person can see their own value and have that attitude affect their social interactions, job satisfaction, and overall sense of well-being (Prakash, 2020). High self-esteem decreases anxiety, depression, and attention disorders due to the person using more positive coping skills. Finding a purpose in life also acts as a protective factor; it has been found to positively affect depression and anxiety, while contributing a 2.4 percent reduction to the chance of developing Alzheimer’s disease (Prakash, 2020). High self-esteem and having a purpose in life are related to a person’s resilience. Also connected to developing resilience are task-oriented coping mechanisms. People learn to use these methods, practicing them throughout their lives, changing them as needed. The current use of mental health apps provides “on the go” coping mechanisms to people who are socially isolated and to the younger generation (Varela et al., 2021).



LINK TO LEARNING

University of California at San Francisco offers [this resource for recommended, evidence-based apps \(https://openstax.org/r/77copingresources\)](https://openstax.org/r/77copingresources) for wellness and mental health.

In, addition, the use of mindfulness techniques—being fully present in the moment, being aware of surroundings, environment, and senses—is another protective factor that reduces stress and thereby lowers risk factors for mental illness. Examples of mindfulness techniques include meditation and keeping gratitude journals, instead of ruminating about the “what-ifs.”

Spirituality, Morals, and Beliefs

The positive (protective) effects of religion and spirituality include promoting positive behaviors, giving people something to believe in, and offering purpose and meaning. The added benefit of the social connectedness that comes from being part of a religious community enhances positive health behaviors and outcomes. It is an important part of the nursing assessment to ask about a client’s religious practices because this information helps to build interventions around the client’s beliefs in health practices and allows nurses to utilize all of the resources that a client may have at their disposal.



LINK TO LEARNING

This [scoping review of scholarly databases \(https://openstax.org/r/77scopingreview\)](https://openstax.org/r/77scopingreview) identified a need for spiritual care to be included in nursing education programs and in staff development efforts in the workplace. The researchers advocate for inclusion of spirituality into nursing practice and call for enhanced overall awareness of needs of health-care recipients.

Motivation/Achievement

Intrinsic motivation is the incentive to do something for internal or inherent satisfaction. Extrinsic motivation, such as working for a paycheck, does not have the same protective factors as intrinsic motivation. Extrinsic motivation can result in burnout and depression, when external rewards are not achieved. Intrinsic motivation is connected with more positive outcomes, such as well-being, satisfaction with life, and using ethical judgment (Kotera & Ting, 2019, p. 229). Being motivated by setting a goal and then achieving that goal is rewarding and can boost self-esteem, contribute to a feeling of fulfillment, and create a more hopeful outlook on life.

Childhood Attachments, Emotional Stability, Coping Skills

Children raised in loving environments are more likely to have good attachments to their parents or primary caregivers. This attachment decreases the likelihood of the child developing aggressive behavior and promotes their ability to express empathy as compared with children who were maltreated (FRIENDS National Center, 2023). The use of positive coping skills to overcome adversity combines cognitive and behavioral techniques that may change over time as the person experiences different stressors (Varela et al., 2021). These strategies are considered either task-oriented or emotionally oriented and each has a different effect on the person's ability to get past the stressor. Being able to have a variety of positive coping skills to use when life gets stressful enhances emotional stability.

External Environment

Social connectedness has been shown to reduce feelings of mental health stigma, to increase feeling supported, and to enhance the use of positive coping skills (Prakash, 2020). The support provided by a social milieu is helpful throughout the span of life. It is also helpful when someone is diagnosed with a mental illness because it provides the support needed to make the person feel validated as they navigate a life that can sometimes be very difficult. Without social support and connectedness, people become lonely, which has been shown to cause detrimental outcomes in people with depression, anxiety, bipolar disorder, and schizophrenia (Prakash, 2020).

Safety, Security, Caregiver Support

Having the support of those around you is an important protective factor. People need to feel secure in their relationships with family, friends, and caregivers. The safety and security they feel from a healthy, nurturing relationship provide a lifeline in times of mental distress. Sometimes just knowing there is one person to depend on makes a difference in being able to overcome a stressful life event. For example, children who are allowed to take risks with the guidance of a parent learn how to be self-sufficient in a safe environment (Prevention United, n.d.).

Participation in Sports, Clubs, Activities, or Groups

Participation in sports, clubs, activities, or groups is a social support that gives the client the feeling of being involved with others. Plus, being physically active releases the “feel good” brain chemicals called endorphins. Endorphins can help reduce depression, stress, and anxiety (Cleveland Clinic, 2023). Doing enjoyable things, such as laughing with friends, listening to music, meditating, and doing volunteer work also releases endorphins.

When encountering stressful situations, the Mayo Clinic (2023) suggests joining mental health support groups. These groups provide a safe and supportive environment in which to meet other people/families going through similar situations. They also provide education and online resources that offer information to help mitigate the risks of normal stress becoming a psychiatric condition. Such groups are provided by the National Alliance on Mental Illness (NAMI), which offers fact sheets and search features for local support groups, and the Department of Veterans Affairs (VA), which offers resources for coaching and connecting.

Access to Medical Care

Having access to health care is a protective factor when it comes to mental illness. According to Mental Health America (2023), barriers to accessing medical care include lack of mental health coverage in a client's private insurance plan, the large number of clients assigned to each mental health provider, and not being able to afford treatment. Public assistance programs, including health insurance, are examples of protective effects of access to medical care; they lift the barrier people have to getting appropriate prenatal care, thus decreasing the risk of a non-live birth (Witt et al., 2012). An additional benefit of being enrolled in a public health program is access to the Special Supplemental Nutrition Program for Women, Infants, and Children (WIC), which will increase prenatal and postnatal nutrition and overall wellness. The support that these people gain through such programs can help to reduce stress and anxiety during pregnancy.

Nursing Implications Associated with Factors of Risk or Protection

As the nurse completes a full assessment of the client, using purposeful questions and active listening are important parts of the process. The nurse must maintain a nonjudgmental attitude during the assessment (Higgins et al., 2015). The assessment builds a rapport between the nurse and client, creating an environment of trust. This is **professional intimacy** in the therapeutic nurse-client relationship, wherein the nurse gains an understanding of the client through respect and empathy, with compassion and recognition of the client's experience. Higgins et al. (2015) explains that the nurse uses three approaches to gather information: unstructured clinical judgment, assessment tools, and structured clinical judgment. Unstructured clinical judgment includes using critical thinking skills, observation, and intuition. Using standardized assessment tools is part of the mental health nursing process that provides insight into risk factors. Structured clinical judgment entails using a combination of the first two approaches to get an overall picture of the person's risk and protective factors.

A psychosocial assessment is a component of the nursing assessment process that obtains additional subjective data to detect risks and identify treatment opportunities and resources. Learning about the client's risk and protective factors will guide the development of appropriate interventions and outcomes. Agencies have specific forms used for psychosocial assessments that typically consist of several components (Glasner, Baltag, & Ambresin, 2021; GW School of Medicine & Health Sciences, n.d.):

- cultural assessment
- reason for seeking health care (i.e., "chief complaint")
- thoughts of self-harm or suicide
- current and past medical history
- current medications
- history of previously diagnosed mental health disorders
- previous hospitalizations
- educational background
- occupational background
- family dynamics
- history of exposure to psychological trauma, violence, and domestic abuse
- substance use (tobacco, alcohol, recreational drugs, misused prescription drugs)
- family history of mental illness
- coping mechanisms
- functional ability/activities of daily living
- spiritual assessment

Risk Assessment

The availability of standardized web-based or paper risk assessment tools can make the risk assessment an easier task for the health-care provider or nurse. Ayhan and Ustun (2021) identified eighteen risk assessment tools that have been developed since 1970. While these individual tools have all been developed to measure levels of risk, each one is based on the client population being assessed. Clinicians must be comfortable with using the correct tool for the environment in which they work, whether it be in-hospital treatment, community care, or forensic psychiatry.

Strengths Assessment

An important question to ask is, "What positive coping skills have you used in the past to get through difficult times?" Assessing and reminding a client of their strengths is an integral part of instilling hope in the client. Who are their support persons? What do they see as their personal strengths? Engaging the client in thinking from a positive standpoint begins the trajectory toward reaching healthy treatment outcomes.



LINK TO LEARNING

The QSEN competency client centered care includes care of a client with a psychiatric-mental illness. The [Client Centered Care competency knowledge, skills, and attitudes \(KSAs\) \(https://openstax.org/r/77KSAs\)](https://openstax.org/r/77KSAs) are expected of

the student nurse as they transition to practice as a licensed nurse. The table serves as a resource to guide curricular development in formal academic nursing programs.

1.3 Mental Health Stigma

LEARNING OBJECTIVES

By the end of this section, you will be able to:

- Discuss stigma surrounding psychiatric-mental health and illness
- Explain the importance of representation and acceptance of mental health in the community
- Describe the nurse's role in stigma reduction

According to the National Alliance on Mental Illness (NAMI) (n.d.-a), “One in 5 Americans is affected by mental health conditions” (para 4). Due to the stigma surrounding mental illness, people delay getting treatment for an average of eight to ten years and less than half will ever seek help (NAMI, n.d.-a). Stigma affects the quality of life for the millions of people with mental illness. Nurses are in a position and have an obligation to educate the public about mental illness to reduce stigma, to help create positive change through engagement in public health advocacy efforts, and to model compassion for those affected by mental illness.

Stigma toward Mental Illness

Despite a recent focus on mental health in the United States, there are still many harmful attitudes and misunderstandings surrounding mental illnesses that lead people to ignore their mental health and make it more difficult for them to reach out for help (Centers for Disease Control and Prevention, 2021a; Corrigan & Watson, 2002). The term **stigma** refers to a cluster of negative attitudes and beliefs that motivates the general public to fear, reject, avoid, and discriminate (Figure 1.6). In this case, the discrimination is against people with mental health disorders (SAMHSA, n.d.-b).

Self	Public	Institutional	Affiliated
Internalizing negative stereotypes related to mental illness	Endorsing negative stereotypes about others with mental illness	Maintaining policies that can lead to decreased access to services for people with mental illness	Being affected by stigma directed at another person with mental illness

FIGURE 1.6 There are four main types of stigma surrounding mental health issues. (attribution: Copyright Rice University, OpenStax, under CC BY NC-SA 4.0 license)

Estimates report that nearly two-thirds of people with diagnosable mental health disorders do not seek treatment due to the stigma of mental illness. The *U.S. Surgeon General's Report* in 1999 was a milestone report that sought to dispel the stigma of mental illness and its impact on those seeking care (Hegner, 2000). This was the first surgeon general report to discuss mental health and mental illness, the first to put mental health in the spotlight. The National Alliance on Mental Illness (NAMI) seeks to improve the lives of those with mental illness and reduce stigma through education, support, and advocacy. NAMI encourages people to share their stories to discredit stereotypes, break the silence, and document discrimination (Abderholden, 2019).



PSYCHOSOCIAL CONSIDERATIONS

Stigma

To fully understand the impact of stigma, it is important to understand several types: self-stigma, public stigma, and institutional stigma. There is also a fourth type of stigma relating to the caretakers of people with mental illness, internalized stigma or affiliated stigma (Kaggwa et al., 2023). Public stigma includes negative or discriminatory attitudes that individuals have about mental illness. Self-stigma involves negative attitudes, including internalized shame, that individuals with mental illness have about their own condition. Institutional stigma is more systemic, involving government and private organization policies that limit opportunities for individuals with mental illness.

Implicit and Explicit Bias

The term **implicit bias** refers to prejudice evidenced in attitudes beyond consciousness or control (Stull et al., 2013). Implicit bias may be automatic and unintentional. Bias against those with mental health disorders comes up in communities, within the health-care sector, and in those with mental illness themselves. Each person's social conditioning affects their implicit bias (Cherry, 2023).

The term **explicit bias** refers to attitudes people are aware of, endorse, and communicate (Vela et al., 2022). Explicit bias interferes with inclusion, equity, and access in health care and negatively affects communication and teaching (Vela et al., 2022). Researchers examining bias in health care call for systemic change to address both implicit and explicit bias (Vela et al., 2022). It is important to work with and advocate for people who have mental illness toward more support and better outcomes for this population. Positive attitudes increase the desire to help and influence behaviors, which is especially important in those who are caring for people with mental illness.

Prejudice

The term **prejudice** refers to the beliefs, thoughts, feelings, and attitudes someone holds about a group. A prejudice is not based on personal experience; instead, it is a prejudgment, originating outside actual experience. While prejudice is based in beliefs outside of experience, experience can lead people to feel that their prejudice is confirmed or justified. This is a type of confirmation bias. For example, if someone is taught to believe that a certain ethnic group has negative attributes, every negative act committed by someone in that group can be seen as confirming the prejudice. Even a minor social offense committed by a member of the group, like crossing the street outside the crosswalk or talking too loudly on a bus, could confirm the prejudice.

Prejudice—as well as the stereotypes that lead to it and the discrimination that stems from it—is most often taught and learned. The teaching arrives in many forms, from direct instruction or indoctrination, to observation and socialization. Movies, books, charismatic speakers, and even a desire to impress others can all support the development of prejudices.

Discrimination

Though prejudice refers to biased thinking, **discrimination** consists of actions against a group of people. Discrimination can be based on race, ethnicity, age, religion, health, sexual orientation, gender identity, or disability. For example, discrimination based on race or ethnicity can take many forms, from unfair housing practices, such as redlining, to biased hiring systems. Overt discrimination has long been part of U.S. history. In the late nineteenth century, it was not uncommon for business owners to hang signs that read, “Help Wanted: No Irish Need Apply.” And Southern Jim Crow laws, with their “Whites Only” signs, exemplified overt discrimination that is not tolerated today.

Discrimination can be intentional in the way that someone else makes a derogatory comment about another's mental illness or treatment, or it can be unintentional when someone is afraid to be close to a person with mental illness due to their lack of understanding about the disease (Mayo Clinic, 2023). This type of discrimination extends to primary caretakers of people with mental illness who may feel judged and unsupported by society (Kaggwa, 2023). Discrimination presents barriers for people with mental health conditions obtaining “good jobs, safe housing, satisfactory health care, and diverse social interactions” (Stull et al., 2013, para 1).

Representation and Acceptance of Mental Health in the Community

Despite the number of people who experience mental illness in any given year, stigma continues to be a social problem in the United States (Ma & Nan, 2018). People with mental illness have ongoing problems finding jobs and affordable and safe housing and are faced with the public's fear of being in close contact with them. Media campaigns can have a positive effect on changing public attitudes. One such campaign is Bring Change 2 Mind hosted by Glenn Close. This program has provided educational public service announcements about mental health/illness since 2009 (Bring Change 2 Mind, 2023).

Gender differences play a role in acceptance of mental illness. In 2018, Ma and Nan found that females are more likely to have positive attitudes toward and empathy for stigmatized groups than their male counterparts. Males have more often been found to want to deal with mental health issues on their own due to fear of stigma (Coveney, 2023).

Culture and Mental Health

Cultural and ethnic considerations are paramount in perceptions of mental health. Culture and ethnicity not only influence the way the disease develops, but also the way it is expressed, accepted, and understood (Mizock & Russinova, 2013). “The explanatory model a culture has for mental illness can both enhance and reduce stigma faced by people with serious mental illness” (p. 231). Cultural values and beliefs affect how a person views certain ideas or behaviors. In the case of mental health, it can determine whether or not the individual seeks help, the type of help sought, and the support available. Every individual has different cultural beliefs and faces a unique journey to recovery. In general, historically marginalized communities in the United States are less likely to access mental health treatment, or they wait until symptoms are severe before seeking assistance (Mental Health First Aid USA, 2019).

Four ways that culture can impact mental well-being are (Mental Health First Aid USA, 2019):

- **Cultural stigma:** Every culture has a different perspective on mental health, and many cultures have a stigma surrounding mental illness. Mental health challenges may be considered a weakness and something to hide, which can make it harder for those struggling to talk openly and ask for help.
- **Describing symptoms:** Culture can influence how people describe or feel about their symptoms. It can affect whether someone chooses to recognize and talk openly about physical symptoms, emotional symptoms, or both. For example, members of the Amish community are typically stoic and endure physical and emotional pain without complaining.
- **Community support:** Cultural factors can determine how much support someone gets from their family and community when it comes to mental health. Because of existing stigma, it can be challenging for individuals to find mental health treatment and support. Globally, there are different beliefs and practices related to the care of mental health problems. Some communities offer support to those with mental illness and others shun or ignore its presence. The treatment received by those defined as mentally ill or disabled varies greatly from country to country.
- **Resources:** When looking for mental health treatment, it can be difficult to find resources and treatment options that take into account a specific culture's concerns and needs.

Communication problems interfere with the spread of mental health knowledge from culture to culture when specific descriptors of diagnoses are not expressed the same ways in different cultures. A solution to this is the use of Transnational and Inclusive Mental Health De-stigmatizing Education (TIMHDE). This concept involves people from all different continents, cultures, and lived experience working together to establish a mutual way to present educational content about mental health (Illingworth, 2021).

Barriers to Mental Health Care

Barriers to mental health care vary. There can be barriers that the client places on themselves by not wanting to admit that they need help. Barriers can also be connected to fear of stigma from friends and family, cultural biases, not having health insurance to cover the costs of treatment, and not having adequate information about the mental health resources available in their community (Heath, 2017).

For those who live in rural areas, finding treatment sources is also not always easy. There may be limited options, especially in rural areas and low-income urban areas; waiting lists; poor quality of care available for indigent clients; and financial obstacles, such as co-pays, deductibles, and time off from work. There is also the consideration that in small rural communities where everybody knows everyone, people may fear getting the help they need because of the stigma associated with mental health care.

Availability, accessibility, and acceptability (the stigma attached to mental illness) are all problems in rural areas. Approximately two-thirds of those with symptoms receive no care at all (U.S. Department of Health and Human Services, 2005; Wagenfeld, Murray, Mohatt, & DeBruin, 1994). At the end of 2013, the U.S. Department of Agriculture announced an investment of \$50 million to help improve access and treatment for mental health problems as part of the Obama administration's effort to strengthen rural communities.



LINK TO LEARNING

The Rural Health Information Hub provides [data and resources, including a Mental Health Toolkit \(https://openstax.org/r/77MHToolkit\)](https://openstax.org/r/77MHToolkit) for communities in rural areas of the United States. The site is supported by the Health Resources and Services Administration of the U.S. Department of Health and Human Services.

Veterans and military personnel make up a large portion of those who encounter barriers to receiving mental health care. Despite the implementation of post deployment screening to help identify signs and symptoms of mental health problems in veterans returning from Iraq and Afghanistan, stigma is still present in the military community (Warner et al., 2011; Brown & Hardey, 2023). Many veterans remain hesitant to disclose their symptoms for fear of it negatively affecting their reputation. Those in the military are trained to be tough, so they fear that “members of my unit might view me differently,” and “It would hurt my career” (para 13).

Nursing Implications

Stigma and negative attitudes toward mental illness can still be found among nurses. Several studies from a variety of countries indicated that health-care professionals can be classified in three categories in relation to stigma, including “stigmatizers,” “the stigmatized,” and “de-stigmatizers.” *Stigmatizers* refer to nurses in medical settings with stereotypical attitudes toward clients with mental illnesses, psychiatric-mental health nurses, and/or psychiatry. Nurses classified as *the stigmatized* have mental health disorders or perceive stigma regarding their roles as psychiatric-mental health nurses. *De-stigmatizers* actively work to reduce stigma surrounding mental health disorders. The authors of the studies found that many nurses share commonly held stereotypical beliefs portrayed in the media. For example, clients with mental health disorders have been portrayed in the media as dangerous, unpredictable, violent, or bizarre, and these portrayals can cause fearful attitudes. Nurses in the studies were concerned about inadvertently saying or doing “the wrong thing” or “setting off” uncontrollable behavior. Many nurses in general medical settings felt they lacked the skills to manage behavioral symptoms of clients with mental health disorders. The authors of the review reported that their findings support additional mental health education for entry-level nurses and practicing nurses to enhance their knowledge base on mental health (Ross & Goldner, 2009).



REAL RN STORIES

Nurse: Lenore, MSN, RN-BC

Years in Practice: 20 years

Clinical Setting: Psychiatric-mental health nurse

Geographic Location: Texas

As a board-certified psychiatric-mental health nurse and university-level instructor, I find it important to teach my nursing students the importance of accepting all people despite what some might think is a derogatory aspect of their being (i.e., diagnosed with a mental illness). In my first class of the semester, I always talk about stigma and show two videos available through Bring Change 2 Mind. The first video “Grand Central Terminal” lets the viewer meet people talking about the diagnosis printed on their white t-shirt. The second video features Ron Howard, who directed “Grand Central Terminal,” sharing why he felt this story was important to tell. After we watch these videos together, I open up the floor for conversation. I ask if seeing visual representation of stigma helps them to better understand this concept. I ask if anyone in class would like to share about a time they may have felt stigmatized. I explain that what happens in class is akin to what happens in Vegas—what happens in the classroom stays in the classroom; it is a safe space for sharing. I also tell my students to compare educating others about stigma to dropping a pebble into a body of water and watching the ripples extend. Much like the ripples in the water, each person they educate has the potential to share that knowledge with others, in effect decreasing the stigma toward mental illness one person at a time.

Nursing Interventions for Client Care

Nurses can reduce stigma and advocate for a client’s needs and dignity by establishing a therapeutic nurse-client

relationship. A therapeutic nurse-client relationship is essential in all settings, but it is especially important in mental health care where the therapeutic relationship is considered the foundation of client care and healing. Although nurse generalists are not expected to perform advanced psychiatric-mental health nursing interventions, all nurses are expected to engage in compassionate, supportive relationships with their clients (Ross & Goldner, 2009). This is even supported in the *Nursing: Scope and Standards of Practice* (American Nurses Association, 2021).

Nurses' Self-Care

Nurses spend a lot of time and energy caring for others. Regis College's 2023 article about self-care tips for nurses highlights five problems that nurses face: "Stress, burnout, bullying, exhaustion, and inadequate sense of support from employers" (para 4). Self-care is an important part of being able to take care of others. If you do not take time to take care of yourself, you will have nothing left to give your clients. Some ideas for self-care include maintaining a sleep schedule as a way to recharge the body, recognizing when to talk to someone about your feelings, making time for exercise, maintaining a healthy diet, and practicing mindfulness (Regis College, 2023). When at work, nurses should make a point of taking breaks, eating lunch, mentoring other nurses (this provides an environment of support), and sharing a laugh with coworkers. In the psychiatric-mental health environment, it is especially important to practice self-care as the clients are often at their most acute level, staffing ratios can be low, and there is the constant need to be vigilant in maintaining a safe environment. These factors increase the stress level of the nurses.

The psychiatric-mental health nurse is taught to be self-aware of their own attitudes toward others and triggers that may occur with certain clients. This self-awareness can help decrease possible stigmatization. For example, there may be a client who reminds the nurse of an abusive family member. In this case, the nurse recognizes the trigger and utilizes reflection and possibly seeks peer or mentor support. By admitting that there are preexisting thoughts about a certain type of client, the nurse is acknowledging that they may not be able to provide nonjudgmental care and reach optimum treatment goals for that client. The role of being a nurse is very rewarding and exhausting all in one. Taking care of self is just as important as the care provided to clients.



LINK TO LEARNING

Are you or someone you know affected by stigma? Take [this quiz at NAMI \(https://openstax.org/r/77stigmaquiz\)](https://openstax.org/r/77stigmaquiz) and learn more about being stigma free.

1.4 Mental Health Recovery and Wellness

LEARNING OBJECTIVES

By the end of this section, you will be able to:

- Discuss principles central to the recovery model
- Identify stages of psychiatric-mental health recovery
- List strategies for psychiatric-mental health wellness

Recovery in mental health is a nonlinear process. According to Mental Health America's 2023 survey, over half of the people in the United States who have mental health disorders, and a staggering 93.5 percent of people with substance use problems, do not get treatment (para 2). People can get better, but they need to understand what resources are available. Health-care providers can educate the individual, families, and communities about the steps of recovery. The goal is to instill hope that with support, the client can maintain wellness throughout their lifetime. Hope is the basis of recovery (SAMHSA, 2023).

Principles of the Recovery Model

Mental illness is treatable. Research reveals that people with mental illness can get better, and many recover completely (Centers for Disease Control and Prevention, 2021a). The majority of individuals with mental illness continue to function in their daily lives. The term **recovery** refers to a process of change through which individuals improve their health and wellness, live a self-directed life, and strive to reach their full potential (Center for Substance Abuse Treatment, 2014). Dimensions that support a life in recovery include the following:

- Health: Overcoming or managing one's disease(s), as well as living in a physically and emotionally healthy way
- Home: Having a stable and safe place to live
- Purpose: Participating in meaningful daily activities, such as a job, school, volunteerism, family caretaking, or creative endeavors, and the independence, income, and resources to participate in society
- Community: Enjoying relationships and social networks that provide support, friendship, love, and hope

There are ten principles of recovery and five stages of recovery. According to the American Psychological Association (2012), the ten principles of recovery are:

1. Self-direction: The journey to recovery is determined by the person recovering.
2. Individualized and person-centered: The chosen road to recovery should be tailored and customized to the client in terms of ability, background, inclinations, and assets.
3. Empowerment: Clients participate in the decision-making process.
4. Holistic: Recovery involves the entire person, including mind, body, spirit, and community.
5. Nonlinear: Recovery is not linear and may entail setbacks. It involves continuing to grow and learning from the setbacks.
6. Strengths-based: Recovery takes what clients are already good at and enhances it.
7. Peer support: Support from others enriches recovery.
8. Respect: Recovery requires acceptance by clients' communities, families, peers, and health-care providers.
9. Responsibility: Clients must take ownership of their own recovery path.
10. Hope: The drive to recover stems from hope of getting better and knowing that it is possible (p. 5).

Additionally, participation in mutual aid groups can be one pathway to recovery. Such groups include twelve-step programs, like Alcoholics Anonymous (AA), Narcotics Anonymous (NA), or Gamblers Anonymous (GA), that provide support for addictions and compulsions. The twelve steps are guidelines for persons to acknowledge and surrender their addiction, and to draw strength from a higher power, as they personally conceptualize that to be.

Hope

Hope is the foundation of the recovery process. Hope in this context is the belief that one can overcome their challenges (SAMHSA, 2023). The nurse helps clients increase their sense of hope by looking beyond the current stressful situation and toward a healthier future. The nurse helps the client identify their “strengths, talents, coping abilities, resources and inherent values” (para 3). Hope is “the catalyst for change” (Laranjeira & Querido, 2022, p. 2). By using a recovery-oriented approach, nurses create a feeling of hope even when the client feels hopeless (Dallum et al., 2015).

Self-Determination, Self-Management, and Empowerment

Recovery is centered around the client developing their own autonomy in making choices about treatment and resources used to achieve their self-directed goal (SAMHSA, 2012). Empowerment begins from the moment a person makes the decision to get treatment. With self-determination, a support system, and learning what feels right in terms of treatment options, a person's sense of empowerment grows. The adage “knowledge is power” is true in recovery. It is here that **psychosocial rehabilitation** comes into play; it helps individuals develop the social, emotional, and intellectual skills needed to live happily with the smallest amount of professional assistance manageable.



CULTURAL CONTEXT

Cultural Views of Drug Use in China

Managed by the courts, compulsory detoxification centers in China have been criticized for human rights violations. However, community-based treatment and voluntary treatment with medical management also exist in the country. Yang and Giummarra (2021) propose evidence-based treatment while acknowledging the Chinese population size, history, and culture.

Criminalization of drug use in China may be preventing incorporation of drug treatment into the health-care system (Yang & Giummarra, 2021).

Advocacy

Advocacy comes in two forms: for the client and for education. A nurse can advocate for the client's needs when they are in too acute a condition to be able to make decisions for themselves. As the client begins to feel better, the nurse steps back and begins to let the client advocate for themselves. One of the foundations of recovery-oriented care is creating safe spaces for healing (Solomon, Sutton, & McKenna, 2021). The high numbers of people with mental health and substance use problems who do not seek treatment attest to the fact that education is important. Nurses can advocate for education in communities about the resources available to fight against substance misuse.

CLINICAL JUDGMENT MEASUREMENT MODEL

Generate Solutions: Advocating for Clients

After completing a client assessment and determining a nursing diagnosis, the nurse must then generate solutions. These must be individualized interventions to address the problems the client is currently facing. The client must be involved in the decision-making process as much as possible in order to have the best chance at achieving the projected outcomes. For example, telling a client that they need to eat healthier foods and then removing certain foods that are culturally relevant to that client will likely not be a maintainable intervention. To make this intervention individualized, it might include advocating for their dietary needs, especially if the intervention is occurring in a facility treatment setting.

Five Stages of the Psychiatric-Mental Health Recovery Model

The five stages of recovery include starting treatment, mental illness education, making a change, finding new meaning, and sticking with recovery (Georgetown Behavioral Hospital, 2023) (Figure 1.7). While each stage builds on the prior one, it is important to remember that recovery itself is a nonlinear process. In the first stage, the person realizes that they need help. During the second stage, the person starts gaining education about their illness and coping skills that can be used during the recovery process. The third stage, making a change, refers to the changes that a person needs to make in their lifestyle, friends, and environment in which they live in order to be able to recover and maintain recovery. The fourth stage encourages trying new positive experiences, making new supports, and finding joy in life. The final stage is planning what recovery is going to look like throughout life: treatment, therapy, and medication are examples. Each of these stages helps the person learn about “self-care, determination, and persistence” (para 10).

The 5 Stages of Recovery				
Starting treatment	Learning about mental illness	Making a change	Finding new meaning	Sticking with recovery
Realizes that help is needed	Gains education about illness and coping skills	Commits to making a change	Begins to make new connections and tries positive experiences, finds the joy in life	Does what is necessary to stay in recovery: treatment, therapy, medication, sponsor
1	2	3	4	5

FIGURE 1.7 In this model, the five stages of recovery have a starting point and a future achievement goal. Stage five is the most difficult and may require starting again at stage one. (attribution: Copyright Rice University, OpenStax, under CC BY NC-SA 4.0 license)

Acceptance

Learning to accept oneself is an important part of recovery. It helps people let go of the past and move forward (Linney, 2022). Accepting that they have an illness helps people face mistakes made in the past and adopt a mindset toward future problem-solving. Part of the process is understanding that recovery will have some rocky spots, but with support people in place and the use of positive coping skills, it is possible to get through those tough times.

Nurses and other health-care personnel must also practice acceptance of the clients for whom they are caring.

Realizing that the client is a human who comes with a history, individual needs, strengths, and values will improve the nurse-client relationship (Solomon, Sutton, & McKenna, 2021).

Insight

As the individual and family become more educated about mental illness and substance misuse (psychoeducation), they begin to gain insight into how their lives are affected by it. Being taught that there are peer support resources available in the community can be invaluable in making the individual feel less alone. Peer support services (see [6.3 Peer Support](#)) provide the individual with a sense of belonging and acceptance (Dell, Long, & Mancini, 2021). Listening to the experiences of the peer support people can help individuals build insight into their own illness and recovery. Family education helps family members to be more understanding, supportive, and can increase the family bond.

Action

One nursing intervention to help individuals take action is to get them involved in illness self-management (ISM). Such programs are meant to help the individual design their recovery in a way that works best for them in their daily lives. Wellness Recovery Action Plan (WRAP) is one of the most often suggested ISMs due to the way it “enhances recovery, self-advocacy, and hope” (Petros, & Solomon, 2021, p. 631). WRAP is led by a trained peer support person and usually consists of a small group that meets over eight to twelve sessions. Instilling hope is the basis for this ISM. Over the course of the program, participants learn how to enhance or maintain their wellness through self-reflecting, being proactive, recognizing triggers, and setting up plans for crisis intervention (Petros & Solomon, 2021).

Self-Esteem

Self-esteem has three parts: (1) the positive and negative thoughts people have about themselves, (2) how people rate their self-worth, and (3) evaluating their own abilities and personal characteristics (Hasani, Aung, & Mirghafourvand, 2021). Low self-esteem is associated with higher rates of depressive symptoms. On the other hand, if a person has a high level of self-esteem, they are more likely to be able to manage stressful events successfully.

The inclusion of self-esteem in Maslow’s hierarchy of needs supports the importance of fulfilling this need in one’s life (Family Addiction Specialist, n.d.). The recovery process helps individuals increase their self-esteem as they begin to rebuild relationships, jobs, and their health.

Healing

Healing is not immediate. It takes time, patience, and education. The medical model for mental health focuses on biology while recovery-oriented mental health focuses on holistic healing (Chisholm & Petrakas, 2021). Medication management can be a viable part of the healing process for people with severe mental illness (SMI) as it can decrease symptoms and help them feel better (Jessell & Stanhope, 2022). Collaboration between the client and the prescriber ensures the client’s autonomy in discussing medication options, side effects, and contributions toward the person’s healing. Healing is not a cure, but is a chance to live a better life than they did before they recognized their illness and entered into recovery. The recovery process involves the client and their families. The hope is that with education about the illness, family members, too, can heal, thus improving perceptions they may have developed about the illness and their loved one (Galimidi, & Shamai, 2022).

Meaning

Finding meaning in life provides individuals protection from stressful situations by helping them to effectively cope and move beyond that challenge (Prakash et al., 2020). Going through the lows of addiction or severe mental illness is sometimes the catalyst for making a positive change (Family Addiction Specialist, n.d.). It is often at the lowest point that an individual finds their purpose to move forward. The individual finds meaning from realizing that they need to make positive changes in their lives.

Strategies for Psychiatric-Mental Health Wellness

Adopting healthier life choices is the most important factor in recovery and working toward/maintaining psychiatric-mental health wellness. Prakash et al. (2020) suggest that the difference in mental illness and mental wellness is that in illness the focus is on management of the illness while wellness focuses on all the aspects in life that can help people live and participate in the world around them. Some wellness strategies include getting good sleep;

eating a diet that includes a balance of fruits, vegetables, lean meats, fish, and whole grains; limiting caffeine use; exercising three to five days per week for forty-five minutes; and maintaining connections to others.



PSYCHOSOCIAL CONSIDERATIONS

What Is Psychosocial Rehabilitation?

Psychosocial rehabilitation helps individuals develop the skills to live happily with the smallest amount of professional intervention manageable. Psychosocial rehabilitation uses two strategies: coping skills to empower the client to more successfully handle stressful situations and resource development to reduce future stressors. Treatments and resources vary case to case but may include medication management, psychological support, family counseling, vocational and independent living training, housing, job coaching, educational assistance, and social support.

Focus on the Positive

Focusing on the positive, despite what may seem like insurmountable obstacles, changes one's perspective. The mindset of always looking at the negative aspects of life can only have a poor effect on one's mental health and hope for recovery. Encouraging a focus on the positive is one of the interventions that nurses can offer to individuals or as a part of therapeutic group work.

Practice Gratitude

Practicing gratitude is embraced by twelve-step programs and recovery specialists and can be an intervention implemented by psychiatric nurses. Encouraging clients to keep a gratitude journal allows them to sit down at the end of the day and reflect on the things for which they are grateful that day. This changes the focus to positive events, no matter how insignificant they may seem and helps the client realize that even when everything seems dismal, there are things to be thankful for in life.

Connect with Others

Learning to have healthy relationships means understanding that sometimes relationships require boundaries in order to stay healthy. When a person is actively using substances or in the midst of severe mental illness, they often make bad choices about the people with whom they connect socially. These connections can negatively affect their mental health. Healthy relationships offer support by increasing one's sense of well-being (Family Addiction Specialist, 2023). Though it may be a difficult decision to cut ties with people and places, it is a necessary part of staying healthy in recovery. As individuals work on their recovery, they begin to learn that in order to become a healthier version of themselves, they need to find and make new friends.

Maintain Physical Health

Physical health and mental health are often connected. Many people have co-occurring diagnoses. For instance, diabetes and depression often co-occur. Teaching clients the importance of maintaining their physical health is just as important as watching for signs and symptoms of mental illness. Again, a holistic approach to the client's health means that mind, body, and spirit are all connected. There are certain things nurses can teach their clients about maintaining better physical health. These include smoking cessation, recovery from substance misuse, improving their diet and exercise routines, and getting plenty of sleep.

In Kesavayuth, Shangkhum, and Zikos (2022), several studies showed connections between health and certain health factors. One study from Nova Scotia reported a connection between having depression and developing coronary artery disease. This connection is further supported by research showing that people with higher levels of physical health are also more likely to have better mental health.



LINK TO LEARNING

Faces and Voices of Recovery tries to remove barriers to recovery. They believe that overall health can influence recovery. Read their article [The Impact of Wellness on Recovery \(https://openstax.org/r/77recovery\)](https://openstax.org/r/77recovery) for additional information.
